

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°134/MVE-BDBV/25/09/2026

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HIGHLIGHTS (last 24 hours)

• In the last 24 hours, 43 new confirmed cases of Ebola Virus Disease (EVD) have been reported, including 10 deaths.

Community transmission has been reported. The confirmed cases were distributed across the provinces of Ituri (31 cases), North Kivu (9 cases), and Haut-Uélé (3 cases). In addition, 9 deaths were recorded in Ebola Treatment Centers (ETCs).

A total of 474 new samples were analyzed, 43 of which were found to be positive for the Bundibugyo virus, representing a positivity rate of 9.1%.

• No new health zones have been affected in the last 24 hours.

ACCUMULATIVE
CAS
7 989

ACCUMULATIVE
DEATH
CONFIRMED
3 852

LETHALITY
48,2 %

PATIENTS IN
ISOLATION/CTE
828

ACCUMULATIVE
HEALED
2 033

TRACKING RATES
CONTACTS
(Of the Day)
77,8 %



Affected provinces and affected health zones



7 Provinces affected

Uele, Ituri, North Kivu,
South Kivu, Tshopo, Lower Uele and
Sud Ubangi



63 out of 167
health zones affected

(37,7%)

Summary of key points from the last 24 hours

Forty-three (43) new confirmed cases of Ebola Virus Disease (EVD) including **10 community deaths** have been recorded: 31 cases in Ituri, 9 in North Kivu and 3 in Haut-Uélé.

However, **9 deaths** were recorded in the CTEs (5 in Ituri, 3 in Haut-Uélé) and 1 in North Kivu), which brings **the total number of confirmed deaths to 19** at the end of the day on September 25, 2026.

In total, **32 patients** (22 in Ituri, 7 in North Kivu, 2 in Haut-Uélé and 1 in Bas Uélé) were declared cured after obtaining two negative results for Ebola virus disease.

As of September 25, 2026, the DRC had recorded **7,989 confirmed cases, including 3,852 deaths**, representing a case fatality rate of **48.2 %**. Ituri province remained the epicenter, concentrating **76.3%** of reported confirmed cases and **72.1%** of new confirmed cases.

No new health zones have been affected in the last 24 hours.

hours. Thus, **63 out of 167 health zones (37.7%)** have been affected in **7 provinces** since the start of the epidemic. Ituri has 28 out of 36 affected health zones, followed by North Kivu (16/34), Haut-Uélé (7/13), Tshopo (7/23), Bas-Uélé (3/11), South Kivu (1/34) and South Ubangi (1/16).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 25, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	31	6 098	2 794	45,8%	28/36 (77,8 %)
North Kivu	9	1 506	901	59,8%	16/34 (47,1 %)
Haut-Uélé	3	328	135	41,2%	7/13 (53,8 %)
Boss	0	43	16	37,2%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	9	4	44,4%	3/11 (27,3%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Total	43	7 989	3 852	48,2%	63/167 (37,7 %)

Table 2. Distribution of confirmed cases and deaths by province and zone as of September 25, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	6098	2794	45,8%	31	5	5	10
Give	11	1	9,1%				0
Undersand	8	6	75,0%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	201	46	22,9%	4	4	0	4
have	2	2	100,0%				0
Bunia	1692	538	31,8%	0	0	3	3
Ladies	40	9	22,5%				0
Drodoro	15	7	46,7%				0
Fireworks	82	31	37,8%	1	1	0	1
Gethy	7	3	42,9%				0
Flounder	2	0	0,0%				0
Kilo	34	12	35,3%				0
Team	162	96	59,3%				0
Liter	282	140	49,6%	2	0	0	0
Logo	13	5	38,5%				0
Fight	33	8	24,2%				0
distributed	4	1	25,0%				0
Washing	27	9	33,3%	2	0	0	0
Darkness	93	45	48,4%	9	0	0	0
Mangala	346	153	44,2%	4	0	0	0
Mongbwalu	697	326	46,8%	1	0	1	1
Nanny	277	152	54,9%	2	0	0	0
Nizi	765	276	36,1%	2	0	0	0
Legumes	127	37	29,1%				0
Jungle	12	4	33,3%				0

Rwampara	1070	362	33,8%	4	0	1	1
Tchomia	76	25	32,9%				0
A valves*	THAT	493	THAT				0
North Kivu	1506	901	59,8%	9	4	1	5
Me	323	216	66,9%	5	2	1	3
Farm	18	7	38,9%				0
Butembo	256	165	64,5%	2	2	0	2
Ten	1	0	0,0%				0
Kalunguta	50	16	32,0%				0
Cut	553	352	63,7%	2	0	0	0
Source	1	0	0,0%				0
Kyondo	33	20	60,6%				0
Lubero	21	11	52,4%				0
Be careful.	25	11	44,0%				0
Manguredjipa	13	5	38,5%				0
Masereka	28	9	32,1%				0
The museums	121	65	53,7%				0
Mutwanga	11	2	18,2%				0
Oicha	23	14	60,9%				0
Vuh	29	8	27,6%				0
Haut-Uélé	328	135	41,2%	3	1	3	4
Kill Mangbetu	39	16	41,0%				0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	100	43	43,0%				0
Lining	76	35	46,1%	2	0	2	2
Beans	2	1	50,0%				0
Ordinary	107	38	35,5%	1	1	1	2
Boss	43	16	37,2%	0	0	0	0
Bafwasende	7	1	14,3%				0
Kabondo	5	1	20,0%				0
Lubunga	1	0	0,0%				0
Maquis-Kisangani	23	10	43,5%				0
Mangobo	5	3	60,0%				0
Boss	1	0	0,0%				0
Wanie-Rukula	1	1	100,0%				0
South Kivu	3	1	33,3%	0	0	0	0
Miti-Murhesa	3	1	33,3%				0
Bas Uélé	9	4	44,4%	0	0	0	0
Blind	1	1	100,0%				0
Viadana	5	1	20,0%				0
Walk	3	2	66,7%				0
Sud Ubangi	2	1	50,0%	0	0	0	0
Hair	2	1	50,0%				0
Total	7989	3852	48,2%	43	10	9	19

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified by province for September 25, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CTE/CT (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	17	1	18	14	1	2	0	15	13
Bas-Uélé	85	0	85	9	0	76	0	9	0
Ituri	927	66	993	100	64	822	0	158	89
North Kivu	684	35	719	86	35	582	0	121	67
Sud Ubangi	30	0	30	2	0	20	0	2	0
Boss	78	1	79	9	1	69	0	10	1
Total General	1 821	103	1 924 220		101	1 571	0	315	170

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

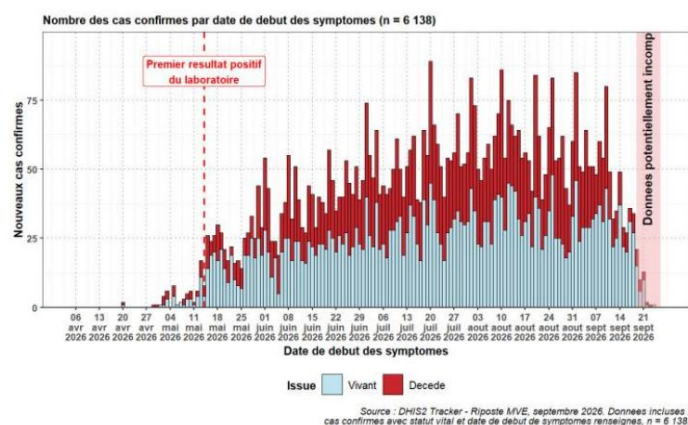
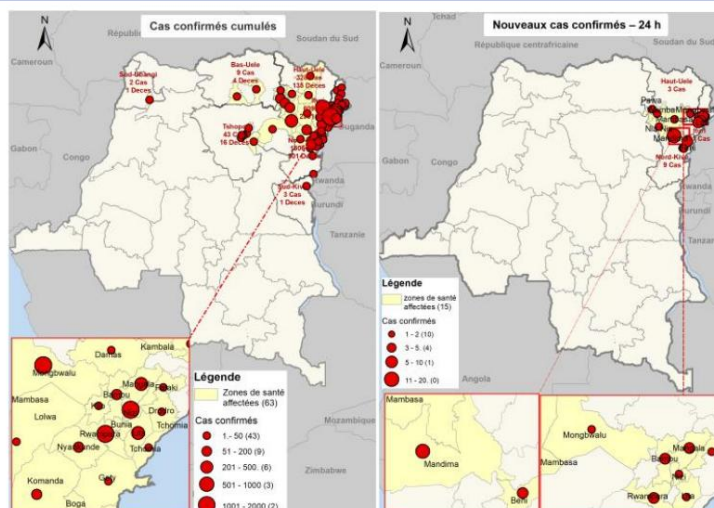


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

Based on the reported data, **1,924 alerts** were raised, and **1,892 (98.3%)** were verified. Of these, **321 alerts were validated as suspected cases** and **315 (98.1%)** were investigated, of which **170** were transferred to CT/CTE (Table 3).

Of the **30,018** contacts currently being monitored, **23,364 contacts were seen** in the last 24 hours, representing a follow-up rate of **77.8%**, lower than the previous day's rate of 79.2%. In South Ubangi, this rate was 92.9% (91/98), 85.2% (121/142) in Bas-Uélé, 79.7% (1,388/1,742) in Haut-Uélé, 79.1% (11,981/15,138) in North Kivu, 77.5% (601/775) in Tshopo, and 75.7% (9,182/12,123) in Ituri.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

For the day of September 25, 2026, three (3) alerts were notified and validated to Different PoE/PoC locations in Ituri province: two symptomatic travelers intercepted at 4

One PoC at Mudzibala (Bunia Health Zone) and one at Uguro (Mahagi Health Zone). These suspects were all isolated and transferred to the CTE (100%).

Table 4. Status of key PoE/PoC indicators by province, as of September 25, 2026

Indicators	Ituri	North Kivu	Boss	Global
Report completeness	96,7% (58/60) 100%	21/21) 100% (18/18)		98,0% (97/99)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	144 142	95 105	21 724	260 971
% of travelers screened	96,2%	98,2%	98,8%	97,2%
% of travelers who washed their hands	97,0%	98,2%	98,6%	97,6%
% of travelers made aware	98,8%	98,2%	100,0%	98,7%
Number of alerts notified	3	0	0	3
Number of validated live alerts (suspected cases)	3	0	0	3
Number of bodies intercepted today % of	0	0	0	0
suspected cases transferred to CTE/CT	100,0%	THAT	THAT	100,0%
% of lifeless bodies swabbed/secured	THAT	THAT	THAT	THAT
% of suspected cases investigated within 4 h	100,0%	THAT	THAT	100,0%

2.2.2. Challenges

In Ituri, the Avakubi PoC is not operational due to insecurity; high refusal rates for screening at the PoCs Nzebi (46%) and Pluto (32%); low capture of alerts in DHIS2 Tracker, insufficient mobility in the ZS of Rwampara, Nizi and Bunia and delayed supply of inputs to some PoCs; in North Kivu, persistent threats against the providers of the PoCs of the Butembo axis and late handling of alerts by the alert cells.

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 31 new positive results** (26 living and 5 deaths) out of 315 new samples received and analyzed (**positivity: 9.8%**).
- **North Kivu: 9 new positive results** (8 living and 1 death) out of 135 samples received and analyzed (**positivity: 6.7%**).
- **Haut-Uélé: 3 new positive results** (2 living and 1 death) out of 10 samples received and analyzed (**positivity: 30.0%**).
- **Tshopo: 13 samples received and analyzed (13 live)** all tested negative.
- **Bas-Uélé: 1 sample received and analyzed (1 live) came back negative.**

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **In North Kivu**, 26 out of 38 treatment centers (68.4%) were opened, 14 out of 40 households were decontaminated (35.0%), and 29 out of 31 health facilities (93.5%) were decontaminated. Four health facilities received infection prevention and control (IPC) kits, 26 were assessed, 243 were monitored, and 463 service providers were briefed, resulting in a 68.8% completion rate (11/16 health zones). Regarding blood and body inspections (BBIs), 54 alerts were received, 51 BBIs were carried out, and 50 bodies were swabbed. Four failed swabbing operations were reported in Beni following the incident that occurred the previous day at the Katula cemetery.
- **In Haut-Uélé**, the only expected ring was opened in Pawa (100%), the household concerned and 8 ESS out of 8 decontaminated; 8 ESS were visited in Isiro, Wamba and Pawa and 59 service providers

They were briefed on triage, hand hygiene, and waste management. As for the EDS (Emergency Response System), 4 alerts were received (Isiro, Pawa, and Watsa), 4 EDS were carried out, and 4 bodies were swabed.

- **In Bas-Uélé**, 1 out of 2 households were decontaminated in Viadana (50.0%) as well as the CSR Limba (1/1); 10 ESS from the ZS of Ganga and Viadana were monitored and 31 out of 33 providers (93.9%) and 4 hygienists were briefed on triage and injection safety.
- **In South Ubangi**, training of service providers on the general aspects of Ebola virus disease (EVD) as well as on standard precautions.

2.4.2. Challenges

In North Kivu : low household decontamination rates (35.0%; 14/40), no IPC kits provided to households with cases (0/14), lack of traceability for some households, insufficient mobility of IPC teams, non-alignment of 86.6% of pillar members on the motivation list, and a need to activate decontamination teams in Biena, Masereka, Kyondo, Mabalako, and Vuhovi. **In Haut-Uélé** : only 3 out of 7 health zones are regularly transmitting information.

their reports, insufficient PCI kits for ESS and households, lack of water source at HGR and CTE in Wamba and delay in the activation of RECO.

2.5. Holistic care

2.5.1. Main actions

- **In Ituri**, 71 new admissions were recorded during the day, compared to 79 discharges, including 22 recoveries. Thus, 385 patients are hospitalized out of 1015 beds (occupancy rate of 37.9%); 360 out of 375 patients (96%) received hot meals, and children under LNPE/SLM were fully covered at the CTE (9/9), at the nursery (9/9), and in the community (33/33); 39 patients are in critical condition.
- **In North Kivu**, 54 new admissions were recorded compared to 68 discharges, including 7 recoveries; 351 Patients are hospitalized, including 271 in standardized facilities, representing an occupancy rate of 70.9%, and 15 patients are in critical condition at the Beni Treatment Center. The 60 patients, 5 accompanying persons, and 156 frontline healthcare providers received hot meals.
- **In Haut-Uélé**, 10 new admissions were recorded against 5 discharges (2 recovered). Thus, 69 patients are hospitalized for 138 beds, representing an occupancy rate of 50%; 67 patients, 132 accompanying persons and 51 PPLs benefited from hot meals.
- **In Tshopo**, four (4) new admissions of suspected cases were recorded with no discharges. Seventeen (17) patients (7 confirmed, including 5 PPL, and 10 suspected) are in isolation in 31 beds, representing an occupancy rate of 54.8%; 7 confirmed, 7 suspected, and 2 accompanying persons received hot meals, as did the 47 frontline providers (100%).
- **In Bas-Uélé**, 2 new admissions of suspected cases, against 1 discharge (1 recovered); 6 patients (1 confirmed in Ganga and 5 suspected cases) remain in isolation in the non-standard CTs of Ganga, Poko, Limba and Buta, while the new confirmed case from Viadana has been transferred to the CTE of Isiro.

2.5.2. Challenges

Treatment remains partially substandard and capacity is limited: **In Ituri**, Ebola Treatment Centers (ETCs) are saturated with suspected cases in Nizi, Mambasa, Lolwa, Kilo, Mangala, and Mandima. **In North Kivu**, 31.2% of hospitalized patients are treated outside of standardized facilities; no beds are available in Butembo and Katwa. **In Tshopo**, the lack of safe blood and limited technical resources persist. **In Bas-Uélé**, the absence of ETCs and ambulances hinders the isolation of cases.

2.6. Vaccination

2.6.1. Main actions

- **In Tshopo**, the shortage of the ERVEBO vaccine persists (0 frozen doses at the hub depot) while demand from the population and PPLs has increased sharply following the death of a doctor on September 24; 500 thawed doses out of the 1,000 planned have been sent to Buta in

awaiting supply from Kinshasa. The provincial total reached 4,628 PPL and TPL vaccinated, or 39.5% of the target (11,703), including 2,266 in Makiso-Kisangani, 599 in Kabondo, 517 in Mangobo, 500 in Bafwasende, 492 in Tshopo and 254 in Lubunga.

- **In Bas-Uélé**, the PPL acceptability survey continues in 4 health zones (Aketi, Ganga, (Titule and Viadana); the cumulative total reached 1,000 people vaccinated, representing 7.6% of the microplan target (13,202), including 550 in Buta (19.2%), 273 in Ganga (20.5%), 71 in Poko, 64 in Titule and 42 in Viadana
- **In Ituri**, Ebola vaccination continued in the Bunia health zone, where 147 People were vaccinated in the last 24 hours, representing a daily coverage of 49% (147/300). Of those vaccinated, 57 were women (38.7%) and 90 were men (61.2%), including 2 breastfeeding women; no pregnant women were vaccinated today. No adverse events following immunization (AEFIs), serious or non-serious, were reported.

2.6.2. Challenges

Total shortage of ERVEBO vaccine in Tshopo (0 frozen doses) in the face of high demand from the population and PPL; coverage still partial of the target in Tshopo (39.5%), low in Lubunga (15%) and in Mangobo (22%); in Bas-Uélé, coverage of only 7.6% and no person vaccinated in Aketi, Ango and Bili, pending doses in accordance with the microplan; in North Kivu, vaccination of PPL not yet started; absence of data from Ituri and Haut-Uélé.

2.7. Continuity of Care

2.7.1. Main actions

- **In Tshopo**, continued supervision of the affected health zones.
- **In Bas-Uélé**, assessment of the availability of inputs in the Bale, Triangle, Congo and CS Baruti; in Tshopo, delivery of UG-PDSS medicines to the 10 priority health zones, evaluation of the SIMAMA Physical Rehabilitation Center and the VITA NOVA Medical Center, development of the recovery plan for the HOZANNA and MUUNGANO Medical Centers, and monitoring of PPLs at the CTM and CTE

2.7.2. Challenges

Major drug shortages in Boma Mangbetu, Isiro, Wamba and Pawa.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In North Kivu**, CREC teams carried out 12,290 home visits (34,860 people), facilitated 119 educational talks (2,917 people), organized 29 mass communication sessions (1,926 people), trained 20 journalists and 40 young leaders; they notified 527 out of 650 expected alerts (81.1%), resolved 11 out of 16 feedbacks (68.8%) and 2 out of 2 incidents, and engaged 1,706 out of 4,750 people (35.9%).
- **In Haut-Uélé**, 462 home visits reached 2,313 people, 146 educational talks were organized (793 people) and 9 mass awareness campaigns (1,215 people); 3 advocacy campaigns (CITBA, revival churches, PROVED); they notified 18 alerts and 1 community death and resolved 23 feedbacks.
- **In Tshopo**, the CREC teams carried out 248 home visits (1,389 people), facilitated 3 talks (134 people), organised 6 mass awareness campaigns (2,348 people) and 4 radio broadcasts; they notified and dealt with 24 community alerts and lifted 2 out of 2 refusals.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **North Kivu**, 17 confirmed cases (100%), 234 confirmed cases under follow-up, 169 new suspects and 346 suspects under follow-up have benefited from SMSPS interventions; 76 households and 64 PPLs have been supported, 7 recovered out of 14 followed in the community (50.0%) and 123 results announced including 17 positive.
- **In Haut-Uélé**, 7 new confirmed cases, 27 confirmed cases under monitoring, and 15 suspected cases benefited from SMSPS interventions, as well as 62 accompanying persons, 40 PPLs, and 33 members of

households; 18 results announced, including 9 positive ones. **In Tshopo**, 7 confirmed and 11 suspected cases received support, and 137 people benefited from psychoeducation.

2.9.2. Challenges

- **In North Kivu**, low SMSPS coverage around the latest cases in 9 health zones and absence of nurseries in 10 health zones; **in Haut-Uélé**, lack of computer equipment, difficult mobility of health workers and poor data reporting in Boma Mangbetu and Pawa; **in Tshopo**, health workers not trained in Wanierukula and Bafwasende.

2.10. Logistics

2.10.1. Main actions

- **In Haut-Uélé**, 5 out of 10 vehicles rented by UNICEF for decontamination were received, one out of 2 isolation tents were erected in Boma Mangbetu, and 2 oxygen concentrators, 2 monitors, and an ECG were delivered to Pawa and Wamba.
- Support for the mobility of response teams; **in North Kivu**, continued construction of the Musienene CTE and cabins at the Mususa PoC; in Tshopo, monitoring of the construction of the Bafwasende CTE and deployment of PCI inputs in 4 ZS; in Bas-Uélé, continued construction of the Buta HGR transit center, with a limited stock of chlorine (25 kg) and fuel (120 litres).

2.10.2. Security

2.10.3. Main actions

- Ensure the safety of response teams in the various affected provinces.

2.11. PSEA (Prevention of Sexual Exploitation and Abuse)

2.11.1. Main actions

- **In Haut-Uélé**, 12 members of the Justice and Peace Commission (AS Tely) and 7 participants in a sewing workshop (AS Nsele) were sensitized in Isiro on the PSEAH, referral mechanisms and the green lines 495555 and 151, and the code of good conduct was popularized among 8 members of the psychosocial pillar in Wamba; the 6 ZS monitored know the reporting mechanism (100%).

2.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 41 of the 119 health zones monitored (34.5%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of alignment by some partners; insufficient mass awareness materials and lack of means of transport and computer kits for the pillar.

MESSAGES CLES

- Active and persistent community transmission is observed in the provinces of Ituri, North Kivu, Haut-Uélé, Tshopo and Bas-Uélé.
- Progress that needs to be consolidated has been made in the area of
Epidemiological investigations: almost all **(98.1 %)** of suspected cases were investigated within 24 hours of notification. However, the
Contact tracing rates remain fluctuating with a slight decrease (77.8% vs 79.2%).
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to rapidly interrupt transmission chains: Rapidly increase reception capacities in ETC/CT in North Kivu and strengthen cross-border surveillance with South Sudan.
- Strengthening preparedness and response to Ebola virus disease in the provinces of River corridor and surrounding areas.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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